DIAMIDE

VIDIA52-var (MY)

DESCRIPTION

Diamide 2.5 mg / 500 mg Film-Coated Tablet
White, oblong, transparent film-coated tablet, shallow convex faces, "HD" embossed and scored at the same face.

Diamide 5 mg / 500 mg Film-Coated Tablet
Oblong, yellow film-coated tablet, shallow convex faces, "HD" embossed and scored at the same face.

COMPOSITION

Each tablet contains:
Diamide 2.5 mg / 500 mg Film-Coated Tablet
Glibenclamide 2.5 mg, Metformin HCl 500 mg

Diamide 5 mg / 500 mg Film-Coated Tablet
Glibenclamide 5 mg, Metformin HCl 500 mg

PHARMACODYNAMICS

Glibenclamide: Glibenclamide is a sulfonylurea antidiabetic. It promotes the increased secretion of insulin from the beta cells of islet tissue in the pancreas by means of a process not yet specifically defined. The overall effect is a reduction in blood glucose concentration only in those patients whose pancreas is capable of synthesizing insulin. Oral antidiabetic medications apparently do not influence the production of insulin by the beta cells but seem to enhance its release from these pancreatic cells.

Metformin Hydrochloride: Metformin is an oral biguanide antidiabetic agent. Its mode of action is thought to be multifactorial and includes delayed uptake of glucose from the gastro-intestinal tract; increased peripheral glucose utilization mediated by increased insulin sensitivity; and inhibition of increased hepatic and renal gluconeogenesis.

PHARMACOKINETICS

Glibenclamide: Glibenclamide is readily absorbed from the gastrointestinal tract, peak plasma concentrations usually occurring within 2 to 4 hours, and is extensively bound to plasma proteins. Absorption may be slower in hyperglycaemic patients and may differ according to the particle size of the preparation used. It is metabolized, almost completely, in the liver, the principal metabolite being only very weakly active. Approximately 50% of a dose is excreted in the urine and 50% via the bile into the faeces.

Metformin Hydrochloride: Metformin Hydrochloride is slowly and incompletely absorbed from the gastrointestinal tract. The absolute bioavailability of a single 500mg dose is reported to be about 50 to 60%, although this is reduced somewhat if taken with food. Plasma protein binding is negligible. It is excreted unchanged in the urine. The plasma elimination half-life is reported to range from about 2 to 6 hours after oral administration.

INDICATIONS

As second-line therapy when diet, exercise and initial treatment with a sulfonylurea or metformin do not result in adequate glycemic control in patients with type 2 diabetes.

CONTRAINDICATIONS

Not recommended in patients with burns, diabetic coma, severe infection, Type 1 diabetes mellitus, ketosis, acute or chronic metabolic acidosis, including diabetic ketoacidosis with or without coma, major surgery, severe trauma, impaired hepatic or renal function; cardiovascular collapse, congestive heart failure, acute myocardial infarction or other conditions leading to hypotension or hypoxemia, infection or gangrene, pregnancy and lactation.

Contraindicated in patients with severely reduced kidney function (GFR < 30 mL/min) and patients with any type of acute metabolic acidosis (such as lactic acidosis, diabetic ketoacidosis).

WARNING AND PRECAUTIONS

- Alcohol should be avoided in patients receiving metformin therapy.
- Glucose urine tests and ketone tests should be monitored.
- To be taken in conjunction with meals. Irregular mealtimes, missed meals, changes in diet may provoke hypoglycemia.
- Lactic acidosis is rare and may occur in significant renal insufficiency.
- Regular renal monitoring, especially in elderly, is needed.
- Avoid use in patients with clinical or lab evidence of hepatic disease.
- Diamide Tablet is not recommended for children.
- Diamide Tablet is capable of producing hypoglycemia or hypoglycemic symptoms, thus, proper patient selection, dosing and instructions are important to avoid potential hypoglycemic episodes.

Lactic acidosis
Lactic acidosis, a very rare but serious metabolic complication, most often occurs at acute worsening of renal function or cardiorespiratory illness or sepsis. Metformin accumulation occurs at acute worsening of renal function and increases the risk of lactic acidosis.

In case of dehydration (severe diarrhoea or vomiting, fever or reduced fluid intake), metformin should be temporarily discontinued and contact with a health care professional is recommended.

Medicinal products that can acutely impair renal function (such as antihypertensives, diuretics and NSAIDs) should be initiated with caution in metformin-treated patients. Other risk factors for lactic acidosis are excessive alcohol intake, hepatic insufficiency, inadequately controlled diabetes, ketosis, prolonged fasting and any conditions associated with hypoxia, as well as concomitant use of medicinal products that may cause lactic acidosis.

Patients and/or care-givers should be informed of the risk of lactic acidosis. Lactic acidosis is characterised by acidotic dyspnoea, abdominal pain, muscle cramps, asthenia and hypothermia followed by coma. In case of suspected symptoms, the patient should stop taking metformin and seek immediate medical attention. Diagnostic laboratory findings are decreased blood pH (< 7.35), increased plasma lactate levels (>5 mmol/L) and an increased anion gap and lactate/pyruvate ratio.

Renal function
GFR should be assessed before treatment initiation and regularly there after [See Section Recommended Dosage]. Metformin is contraindicated in patients with GFR <30 mL/min and should be temporarily discontinued in the presence of conditions that alter renal function [See Section Contraindications].

Metformin may reduce vitamin B12 serum levels. The risk of low vitamin B12 levels increases with increasing metformin dose, treatment duration, and/or in patients with risk factors known to cause vitamin B12 deficiency. In case of suspicion of vitamin B12 deficiency (such as anemia or neuropathy), vitamin B12 serum levels should be monitored. Periodic vitamin B12 monitoring could be necessary in patients with risk factors for vitamin B12 deficiency. Metformin therapy should be continued for as long as it is tolerated and not contraindicated and appropriate corrective treatment for vitamin B12 deficiency provided in line with current clinical guidelines.

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INTERACTIONS WITH OTHER MEDICAMENTS

- Concurrent use with monoamine-oxidase inhibitors, oral anticoagulants, oxyphenbutazone, phenylbutazone and alcohol may increase glibenclamide activity.
- Co-administration of cationic drugs that are eliminated by renal tubular secretion, Furosemide, hyperglycemia-causing and hypoglycemia-causing medications and alcohol have a potential for interaction with metformin activity.
- Blood glucose levels may be increased when used concurrently with corticosteroids, epinephrine, phenytoin, thiazide diuretics and thyroid hormones.
- Co-administration of beta-adrenergic blocking agents may mask symptoms of developing hypoglycaemia thus complicating patient monitoring.
- An increased hypoglycaemic effect has occurred or might be expected with ACE inhibitors, alcohol, allopurinol, some analgesics (notably azapropazone, phenylbutazone, and the salicylates), azole antifungals (fluconazole, ketoconazole, and miconazole), chloramphenicol, cimetidine, clofibrate and related compounds, coumarin anticoagulants, halofenate, heparin, MAOIs, octreotide (although this may also produce hyperglycaemia), ranitidine, sulphinpyrazone, sulphonamides (including co-trimoxazole), tetracyclines, tricyclic antidepressants, and thyroid hormones.

PREGNANCY AND LACTATION

Not recommended in pregnancy and lactation.

MAIN SIDE/ADVERSE EFFECTS

- Gastrointestinal adverse effects including anorexia, diarrhea, dyspepsia, flatulence, nausea, vomiting.
- Prolonged hypoglycaemia has been reported following the ingestion of glibenclamide.
- Cross sensitivity to sulphonamides or their derivatives may occur. Transient visual disturbances may occur at the start of treatment. Reversible leucopenia and thrombocytopenia have been reported but are rare. Agranulocytosis, pancytopenia and haemolytic anaemia have been reported very rarely.
- Treatment with sulphonylureas has been associated with occasional disturbances of liver function and cholestatic jaundice. If hepatitis or cholestatic jaundice occurs, Diamide Tablet should be discontinued.
- Long-term Metformin therapy may cause a decrease of vitamin B_{12} absorption with decrease of serum levels.

Metabolism and nutrition disorders:
Common: Vitamin B12 decrease/deficiency.

OVERDOSE

Clinical features: Hypoglycemia and lactic acidosis.

TREATMENT OF OVERDOSAGES

- For mild to moderate hypoglycemia: Treating with immediate ingestion of a source of glucose and counseling patient to obtain emergency medical assistance immediately.
- For severe hypoglycemia or acute overdose, including coma:
  - Dextrose administration is the basis of treatment of hypoglycemia.
  - Counseling patient to obtain emergency medical assistance immediately.
- For lactic acidosis: Hemodialysis with sodium bicarbonate.

DOSAGE AND ADMINISTRATION

Mode of administration: Oral

Second-line therapy: ***Usual adult dose:***
Initial dose of one tablet 2 times daily with meals.

If necessary, medication can be increased gradually to a maximum daily dose of 20 mg glibenclamide / 2000 mg metformin.

Renal impairment
A GFR should be assessed before initiation of treatment with metformin containing products and at least annually thereafter. In patients at an increased risk of further progression of renal impairment and in the elderly, renal function should be assessed more frequently, e.g. every 3-6 months.

The maximum daily dose of metformin should preferably be divided into 2-3 daily doses. Factors that may increase the risk of lactic acidosis should be reviewed before considering initiation of metformin in patients with GFR <60 ml/min.

If no adequate strength of Diamide is available, individual monocomponents should be used instead of the fixed dose combination.

GFR mL/min	Metformin
60 – 89	Maximum daily dose is 3000 mg. Dose reduction may be considered in relation to declining renal function.
45 – 59	Maximum daily dose is 2000 mg. The starting dose is at most half of the maximum dose.
30 – 44	Maximum daily dose is 1000 mg. The starting dose is at most half of the maximum dose.
< 30	Metformin is contraindicated.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:
Diamide 2.5 mg / 500 mg Film-Coated tablet
Store below 30°C. Protect from light and moisture.

Diamide 5 mg / 500 mg Film-Coated tablet
Store below 30°C. Protect from light and moisture.

Presentation/Packing:
Blister pack of 1x10's and blister pack of 12 x10's.

Product Registration Holder: HOVID Bhd.
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30010 Ipoh, Perak, Malaysia.

Manufactured by: HOVID Bhd.
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